Superficial Basal Cell Carcinoma

Superficial basal cell carcinoma (BCC) is characterized microscopically by buds of malignant basaloid cells extending from the basal layer of the epidermis into the upper dermis. The peripheral cells typically show palisading. Epidermal atrophy may be present, and dermal invasion is usually minimal. This subtype is most commonly found on the trunk and extremities, although it can also occur on the head and neck. A chronic inflammatory infiltrate is often present in the upper dermis.

Morpheaform Basal Cell Carcinoma

Morpheaform (or infiltrative) BCC consists of thin strands or cords of tumor cells embedded within a dense fibrous stroma. The tumor cells are closely packed and may be as narrow as a single cell in thickness. These strands extend deeply into the dermis, often beyond the clinically apparent margins, making the tumor larger than it appears on physical examination. Recurrent BCCs may also exhibit similar infiltrative patterns within scar tissue.

Fibroepithelioma of Pinkus

Fibroepithelioma of Pinkus (FEP) is a variant of BCC that presents histologically as long, interwoven strands of basaloid cells within a fibrous stroma. It shares features with both reticulated seborrheic keratoses and superficial BCC, making diagnosis dependent on careful histologic evaluation.

Differential Diagnosis

Nodular BCC

- Dermal nevus
- Squamous cell carcinoma
- Appendageal tumor
- Dermatofibroma
- Scar
- Seborrheic keratosis

Pigmented BCC

- Nodular melanoma
- Superficial spreading melanoma
- Lentigo maligna melanoma
- Appendageal tumor
- Compound nevus
- Blue nevus

Superficial BCC

- Bowen disease
- Mammary or extramammary Paget disease
- Superficial spreading melanoma
- Single plaque of psoriasis
- Single plaque of eczema

Morpheaform BCC

- Scar
- Morphea

- Trichoepithelioma

Fibroepithelioma of Pinkus

- Skin tag
- Papillomatous dermal nevus
- Fibroma

Treatment

Management of BCC depends on anatomic location, tumor subtype, and histologic features. Treatment options include Mohs micrographic surgery (MMS), standard surgical excision, destructive modalities (e.g., curettage and desiccation, cryotherapy), radiotherapy, and topical chemotherapy.

The best chance for cure is with appropriate initial treatment of primary BCC, as recurrent tumors are more likely to recur again and cause extensive local tissue destruction. An algorithmic approach to management should be followed based on risk stratification.

Mohs Micrographic Surgery (See Chap. 245)

Mohs micrographic surgery (MMS) provides superior margin control compared to standard excision, allowing for maximal tissue preservation while achieving high cure rates. Studies report a recurrence rate of **1%** for primary BCCs treated with MMS, significantly lower than other modalities: - Standard excision: 10%
- Curettage and desiccation (C&D): 7.7%
- Radiotherapy (XRT): 8.7%
- Cryotherapy: 7.5%

For recurrent BCCs, MMS has a recurrence rate of **5.6%**, again outperforming:
- Excision: 17.6%

- XRT: 9.8%
- C&D: 40%

MMS is the treatment of choice for morpheaform BCC, poorly demarcated tumors, incompletely excised lesions, and other high-risk cases.